NCCP 00373 · VERSION 3

Your treatment, explained

Advanced melanoma · NCCP 00373 v3 · Document revised 6 November 2026

> **Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Cobimetinib + vemurafenib — advanced melanoma

Reference treatment cycle

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · cotellic-pi · zelboraf-pi

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Your treatment phases
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and overall plan
Taking or receiving treatment safely
Tests, results and treatment decisions
What to expect between visits
Skin, body changes and daily care
Fertility, pregnancy and breastfeeding
Decisions, support and useful records
After a treatment hold, change or completion
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

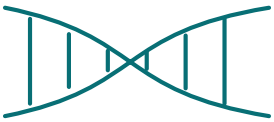
What each medicine does

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

MEK TARGETED TREATMENT

Cobimetinib

Cotellic



Blocks a second growth signal

Cobimetinib inhibits MEK, further along the growth-signalling pathway affected by BRAF. It is combined with vemurafenib. Cobimetinib has a seven-day planned break in each 28-day cycle; vemurafenib continues through that break unless the team holds it.

Blocks a second growth signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

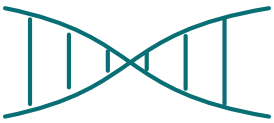
Report new vision changes, muscle pain with dark urine, breathlessness, ankle swelling or unusual bleeding.

Sources: nccp · cotellic-pi

BRAF TARGETED TREATMENT

Vemurafenib

Zelboraf



Blocks an altered BRAF signal

Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report new skin growths, sunburn-like reactions, eye pain or blurred vision, palpitations or fainting.

Sources: nccp · zelboraf-pi

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Eyes / retina

Cobimetinib + vemurafenib

New blurred vision, distortion, blind spots or eye pain can reflect retinal or inflammatory problems. Contact the team promptly rather than waiting for the next cycle.

3 Skin / sun

Cobimetinib + vemurafenib

Rash, photosensitivity and new skin lesions need protection and review. Severe blistering or peeling needs urgent help.

5 Kidneys / fluids

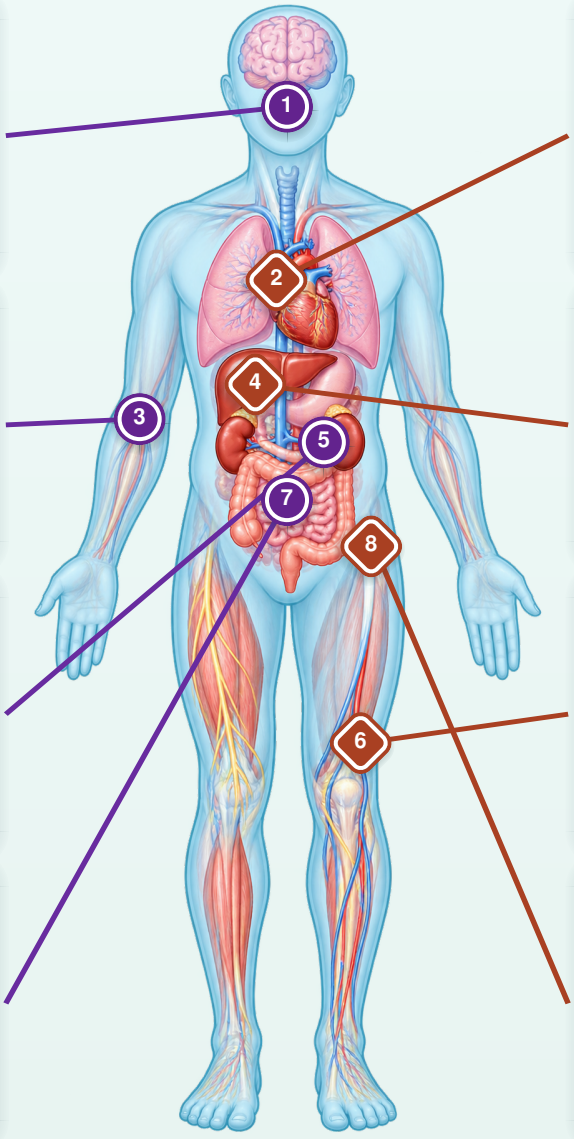
Cobimetinib + vemurafenib

Diarrhoea, vomiting or muscle damage can affect kidneys. Reduced urine or dark urine with muscle pain is urgent.

7 Bowels

Cobimetinib + vemurafenib

Diarrhoea, nausea and abdominal discomfort can cause dehydration. Report persistent symptoms; severe pain or blood needs urgent assessment.



2 Heart function / rhythm

Cobimetinib + vemurafenib

Cobimetinib can reduce heart pumping; vemurafenib can alter rhythm. Breathlessness, swollen ankles, palpitations or fainting need review.

4 Liver

Cobimetinib + vemurafenib

Blood tests can detect liver injury before symptoms. Report jaundice, dark urine or upper-abdominal pain.

6 Muscles

Cobimetinib

Severe muscle pain, weakness or dark urine may signal muscle breakdown. The team may check muscle enzymes and kidneys.

8 Bleeding

Cobimetinib

Unusual bruising, persistent nosebleeds, black or bloody stools, coughing blood or a severe new headache need prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Your treatment phases

THREE WEEKS

Days 1–21: both medicines

Cobimetinib once daily **Vemurafenib twice daily**

Take both medicines according to their separate labels. Review symptoms, tests and dose changes before repeating.

ONE WEEK

Days 22–28: vemurafenib continues

Planned cobimetinib break **Vemurafenib continues twice daily**

Do not stop vemurafenib just because cobimetinib has its planned break. A toxicity hold is a different instruction and must name which medicine is affected.

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00373 v3: current regimen source · EMA Cotellic product information · EMA Zelboraf product information

THE WHOLE TREATMENT JOURNEY

Cobimetinib + vemurafenib — advanced melanoma: treatment and checks

Advanced melanoma

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00373	Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms.	Blood count, kidney/ liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.	Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.	After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00373	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00373	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1

Days 1–21: both medicines

Three weeks

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00373

2

Days 22–28: vemurafenib continues

One week

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00373

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily tablets / capsules	Days 1–21	Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.
Vemurafenib continues; cobimetinib break	Days 22–28	Continue vemurafenib twice daily; no cobimetinib on days 22–28 unless the team has issued a different written plan.

Treatment or check	Days in this window	What happens
Safety and symptom review	Days 1–28	Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.

Schedule and review: Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Holds, missed treatment and phase changes: For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Why these bands stay broad: This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00373 v3: current regimen source ↗ · EMA Cotellic product information ↗ · EMA Zelboraf product information ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–21: At home each day

Usual starting doses are cobimetinib 60 mg once daily on days 1–21 and vemurafenib 960 mg twice daily, about 12 hours apart, continuously. Cobimetinib can be taken with or without food. Avoid regularly taking both daily vemurafenib doses on an empty stomach. Swallow both medicines whole and follow the separate labels. For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Day 22–28: Planned cobimetinib break

Vemurafenib continues twice daily. The seven-day break applies to cobimetinib only. Record any additional treatment hold separately and confirm the next cobimetinib start with the team.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Usual starting doses are cobimetinib 60 mg once daily on days 1–21 and vemurafenib 960 mg twice daily, about 12 hours apart, continuously. Cobimetinib can be taken with or without food. Avoid regularly taking both daily vemurafenib doses on an empty stomach. Swallow both medicines whole and follow the separate labels.

STEP 2

Missed, vomited or held treatment

For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

STEP 3

Appointments and supply

Arrange blood tests, transport, medicine supply and interpreter or carer support. Check who reviews results and how you receive the decision before the next dose.

STEP 4

Paper copies and contacts

Keep the full handbook, three-page summary and wallet card together. The QR code is optional. Ask the team to replace every labelled example contact with your real oncology and out-of-hours details; ask for a new printed plan when treatment changes.

Sources: NCCP 00373 v3: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Supportive medicines and symptom plan

NCCP lists each component as minimal-to-low emetogenic risk; follow local and individual anti-sickness policy. Agree a diarrhoea, fluid-intake and nausea plan; contact the team if symptoms persist or you cannot drink. Check all prescribed, over-the-counter and herbal medicines for interactions before taking them. The seven-day cobimetinib break is planned; extra breaks or restarting doses require the team's instructions.

2 Skin and daily care

Vemurafenib can make skin very sensitive to sunlight. Use UVA/UVB protection, protective clothing and SPF 30 or higher with lip protection. Continue skin checks for new growths or changing lesions. Do not assume combining the medicines removes the need for skin surveillance.

3 Fertility, pregnancy and breastfeeding

The cobimetinib source specifies two effective contraception methods during treatment and for at least three months afterwards; vemurafenib requires at least six months after its last dose. The combined plan therefore needs to cover the longer six-month vemurafenib interval and account for reduced hormonal-contraceptive effectiveness. Discuss fertility and breastfeeding before treatment.

Sources: nccp · cotellic-pi · zelboraf-pi

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Keep the pathology report, original melanoma site, stage and known sites of spread. This guide concerns advanced disease that cannot be removed completely by surgery or has spread; it does not describe a postoperative recurrence-prevention course.

Sources: nccn-patient

The tumour test

A BRAF V600 test helps identify whether BRAF-targeted medicines are an option. A tumour mutation does not by itself mean an inherited family condition. Keep the exact result and ask how it changes your choices.

Sources: nccn-patient

Scans and local treatments

Record the scans used to follow known disease. Surgery or radiotherapy may have a separate role for a particular site; ask for their timing and risks to be added to the overall plan.

Sources: nccn-patient

New changes still matter

Report a new lump, changing mole, persistent skin sore or unexplained symptom. A response at one site does not establish that every other symptom is harmless.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and overall plan

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

The schedule

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Sources: nccp · cotellic-pi · zelboraf-pi

How treatment works

Cobimetinib inhibits MEK, further along the growth-signalling pathway affected by BRAF. It is combined with vemurafenib. Cobimetinib has a seven-day planned break in each 28-day cycle; vemurafenib continues through that break unless the team holds it. Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Sources: nccp · cotellic-pi · zelboraf-pi

Important boundaries

Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. ECG measures rhythm; an echocardiogram or other LVEF test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Sources: nccp · cotellic-pi · zelboraf-pi

Evidence and choices

Median progression-free survival: 12.3 versus 7.2 months (HR 0.58, 95% CI 0.46–0.72). Median overall survival: 22.3 versus 17.4 months (HR 0.70, 95% CI 0.55–0.90). These are group results from that study, not a prediction for you. Direct comparison was against vemurafenib alone. It does not rank this regimen against other BRAF/MEK pairs or immunotherapy.

Sources: cobrim · nccp

TREATMENT REFERENCE · 03

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Administration

Usual starting doses are cobimetinib 60 mg once daily on days 1–21 and vemurafenib 960 mg twice daily, about 12 hours apart, continuously. Cobimetinib can be taken with or without food. Avoid regularly taking both daily vemurafenib doses on an empty stomach. Swallow both medicines whole and follow the separate labels.

Sources: nccp · cotellic-pi · zelboraf-pi

Missed or held treatment

For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Sources: nccp · cotellic-pi · zelboraf-pi

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Sources: nccp · cotellic-pi · zelboraf-pi

Practical arrangements

Know where and when bloods are taken, how results reach the prescriber, and how to obtain the next supply or appointment. Ask for written instructions in a format you can use without a phone.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms.

Sources: nccp · cotellic-pi · zelboraf-pi

During treatment

Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.

Sources: nccp · cotellic-pi · zelboraf-pi

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

Sources: nccp · cotellic-pi · zelboraf-pi

Response and follow-up

After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.

Sources: nccp · cotellic-pi · zelboraf-pi

TREATMENT REFERENCE · 05

What to expect between visits

A quiet treatment day does not rule out later problems.

Act on new symptoms

Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

Sources: nccp · cotellic-pi · zelboraf-pi

Supportive medicines

NCCP lists each component as minimal-to-low emetogenic risk; follow local and individual anti-sickness policy. Agree a diarrhoea, fluid-intake and nausea plan; contact the team if symptoms persist or you cannot drink. Check all prescribed, over-the-counter and herbal medicines for interactions before taking them. The seven-day cobimetinib break is planned; extra breaks or restarting doses require the team's instructions.

Sources: nccp · cotellic-pi · zelboraf-pi

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

Sources: nccp · cotellic-pi · zelboraf-pi

When life is affected

Tell the team about fatigue, pain, appetite, sleep, mood and ability to work or care for others. Practical and supportive care can accompany anticancer treatment; you do not have to wait until treatment ends.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

Skin and sun

Vemurafenib can make skin very sensitive to sunlight. Use UVA/UVB protection, protective clothing and SPF 30 or higher with lip protection. Continue skin checks for new growths or changing lesions. Do not assume combining the medicines removes the need for skin surveillance.

Sources: nccp · cotellic-pi · zelboraf-pi

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Sources: nccp · cotellic-pi · zelboraf-pi

Food, fluids and activity

Keep meals and fluids manageable and follow any individual fluid or diet restrictions. Gentle activity and rest can be adjusted to how you feel. Ask for dietetic or rehabilitation help if intake, weight, strength or mobility is falling.

Sources: nccp-patient

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Sources: nccp · cotellic-pi · zelboraf-pi

TREATMENT REFERENCE · 07

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Contraception and pregnancy

The cobimetinib source specifies two effective contraception methods during treatment and for at least three months afterwards; vemurafenib requires at least six months after its last dose. The combined plan therefore needs to cover the longer six-month vemurafenib interval and account for reduced hormonal-contraceptive effectiveness. Discuss fertility and breastfeeding before treatment.

Sources: nccp · cotellic-pi · zelboraf-pi

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Sources: nccp · cotellic-pi · zelboraf-pi

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sources: nccp · cotellic-pi · zelboraf-pi

Sexual health

Tell the team about sexual symptoms, relationship concerns or practical difficulties with the agreed precautions. Ask for a private discussion and specialist support when needed.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Sources: cobrim · nccp

Support alongside treatment

Ask about symptom control, psychological support, finances, transport, work and help at home. Palliative or supportive care can be used alongside anticancer treatment to improve comfort and function.

Sources: nccn-patient

A person you trust

If helpful, bring a family member or supporter and agree what information may be shared. Ask for an interpreter or accessible information when needed; use teach-back to check the schedule and urgent plan.

Sources: nccn-patient

Your document set

Keep consent information, the three-page summary, complete handbook, treatment card, test plan and real contact details together. The QR opens more information but essential instructions remain available in print.

Sources: nccn-patient

TREATMENT REFERENCE · 09

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.

Sources: nccp · cotellic-pi · zelboraf-pi

Restarting or changing treatment

For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Sources: nccp · cotellic-pi · zelboraf-pi

A treatment summary

Record medicines and doses received, important side effects, admissions, last-dose date, ongoing symptom treatment and the clinician responsible for follow-up. Ask for an updated paper copy for your GP and other treating teams.

Sources: nccn-patient

Your next review

Confirm the next blood test, scan and review, who explains the result, and which symptoms should trigger contact sooner. Keep actual daytime and out-of-hours numbers accessible after treatment stops.

Sources: nccn-patient

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

Median progression-free survival: 12.3 versus 7.2 months (HR 0.58, 95% CI 0.46–0.72). Median overall survival: 22.3 versus 17.4 months (HR 0.70, 95% CI 0.55–0.90). These are group results from that study, not a prediction for you.

Sources: cobrim · nccp

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the treatment schedule?

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

What if a dose is missed or held?

For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Which symptoms need early contact?

Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

Which tests are needed?

Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms. Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks. After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00373 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00373 v3: current regimen source
- EMA Cotellic product information
- EMA Zelboraf product information
- NCCN Guidelines for Patients: Melanoma, 2026 — supplied education reference
- Ascierto et al. coBRIM, Lancet Oncology 2016

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6728/373_V3_Cobimetinib_and_Vemurafenib.pdf

https://www.ema.europa.eu/en/documents/product-information/cotellic-epar-product-information_en.pdf

https://www.ema.europa.eu/en/documents/product-information/zelboraf-epar-product-information_en.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf

https://pubmed.ncbi.nlm.nih.gov/27480103/